TECHNOVATE 2.0 | HEALTHTECH | PS-05

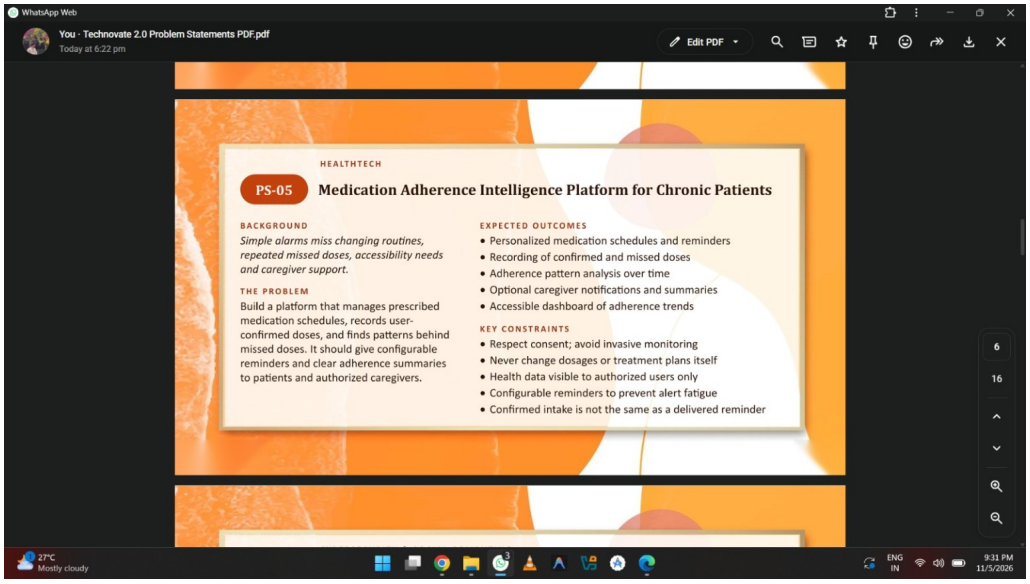
Medication Adherence Intelligence Platform

Product Requirements Document (PRD)

Version	1.0 · Draft for product design and hackathon development
Date	6 November 2026
Product stage	MVP definition and prototype planning
Primary users	Chronic patients and consent-authorized caregivers

Product thesis

Help people living with chronic conditions follow their prescribed medication routines through reliable reminders, explicit dose tracking, understandable adherence insights and patient-controlled caregiver support.



Original PS-05 problem statement supplied by the project team.

CONFIDENTIALITY NOTE: This document is a product planning artifact, not clinical guidance. The platform must not diagnose, prescribe, change doses or advise patients to compensate for missed medication.

Document map

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1. Executive summary and product vision

Product: Medication Adherence Intelligence Platform (working title).
Category: HealthTech / chronic-care support.
Primary platform: Mobile-first patient experience with responsive caregiver and healthcare dashboards.

The platform helps chronic patients manage prescribed medication schedules and record what happened at each dose. It turns dose-event history into transparent adherence summaries and patient-reported patterns. Patients can optionally share selected information with caregivers and revoke access.

The product should not be positioned as another reminder app. Its differentiation is the connected loop: schedule → reminder → patient response → pattern insight → consent-based support.

Product vision

Make consistent medication routines easier to maintain, while giving patients control over their information and making adherence history easier to understand during care conversations.

Design statement

Calm, accessible, low-friction and trustworthy. The patient experience must work for people with limited digital confidence, changing routines, low-end Android devices and intermittent connectivity.

2. Problem definition and opportunity

The supplied PS-05 brief identifies four connected shortcomings: simple alarms do not adapt to changing routines; repeated missed doses are difficult to understand; accessibility and caregiver support are often insufficient; and a reminder being delivered does not prove medication was taken.

Problem statement

Chronic patients need a dependable, accessible way to manage prescribed medication schedules and record dose outcomes. Patients and authorized caregivers need clear, privacy-respecting visibility into adherence patterns so they can provide appropriate support without changing treatment plans.

Why existing approaches fall short

- Reminder-only experiences can become noisy or rigid when a patient's daily routine changes.
- Manual medication setup and repeated confirmation can create friction, especially for people managing several medicines.
- Many dashboards show counts or percentages without explaining whether a dose was confirmed, skipped or simply left unresolved.
- Caregiver visibility can be either absent or too broad; patient-controlled, granular sharing is essential.
- Connectivity, notification permissions, battery restrictions, language and accessibility can undermine otherwise good reminder designs.

Opportunity

Build a reliable adherence record first, then add explainable intelligence on top. The MVP should favor correctness, clear status semantics and user trust over speculative prediction or complex AI.

3. Goals, non-goals and success metrics

Product goals

- Help patients create and maintain accurate schedules based on verified prescription instructions.
- Make it easy to record taken, skipped or unresolved doses.

- Provide a transparent view of adherence over time, including uncertainty.
- Support patient-authorized caregiver access and configurable escalation.
- Detect simple, explainable patterns that can prompt reflection or a conversation with a healthcare professional.

Non-goals

- Diagnosing conditions, recommending treatments or changing medication/dosage.
- Inferring that medication was taken from a reminder delivery, notification open, location or passive surveillance.
- Automatically contacting emergency services or making clinical triage decisions.
- Replacing a clinician, pharmacist, pharmacy dispensing system or electronic health record.
- Claiming improved clinical outcomes before conducting an appropriate evaluation.

North-star metric

Weekly scheduled-dose outcome coverage: the proportion of scheduled dose events that have a clearly recorded outcome (taken, skipped, or explicitly marked not taken) within the defined reporting window. This measures record completeness, not clinical adherence by itself.

Metric	Definition	MVP target / approach
Dose confirmation rate	Confirmed taken events ÷ scheduled dose events	Establish baseline; segment by patient and medication
Unresolved dose rate	Dose events with no recorded outcome by close of window ÷ scheduled events	Trend downward without coercive reminders
Reminder reliability	Delivered reminders ÷ attempted reminders where delivery telemetry is available	Track by device/OS; disclose telemetry limits
Schedule setup completion	Users completing at least one verified schedule ÷ users starting setup	Measure funnel drop-off
Caregiver activation	Patients who grant at least one caregiver permission	Measure opt-in, not as a forced adoption target
Retention	Patients active in week 4 after onboarding	Cohort-based baseline and improvement
Insight usefulness	Patient rating / action taken after an insight	Qualitative validation before optimization

4. Users, personas and jobs to be done

Persona	Needs	Design implications
Patient managing chronic medication	Remember schedules, confirm doses quickly, understand own history, retain control	Simple home screen, large actions, clear statuses, offline access, language choice
Caregiver / family supporter	Know when support is needed without monitoring everything	Explicit invitation, granular permissions, configurable summaries, easy revocation
Healthcare professional (future / limited MVP)	Review patient-approved adherence history	Read-only report export; no treatment modification workflow
Platform administrator	Maintain service, support users and investigate technical issues	Least-privilege operations, audit trail, no routine access to health content

Jobs to be done

- When it is time for a scheduled dose, I want a reminder I can act on quickly so I can keep track of my routine.
- When I cannot take a dose, I want to record that accurately without being shamed or told to double the next dose.
- When my routine changes, I want to adjust reminder timing without accidentally changing the prescribed instructions.

- When I choose to involve a caregiver, I want to decide exactly what they can see and what alerts they receive.
- When I meet my clinician, I want to share a clear history of what I recorded.

5. Product principles and experience strategy

Principle	Product rule
Patient agency	Sharing is opt-in, granular and revocable. No caregiver access by default.
Truthful status	A reminder sent is not a dose taken. Keep scheduled, delivered, acknowledged and confirmed events distinct.
Safety before automation	Never alter treatment instructions or advise on missed-dose compensation.
Low cognitive load	One primary action per reminder; readable text; accessible touch targets; restrained alerts.
Graceful offline behavior	Show cached schedules and allow local dose logging; sync safely when connectivity returns.
Explainable intelligence	Show the dates and events behind a pattern; avoid opaque risk scores in the MVP.
Respectful tone	No guilt, blame, streak pressure or alarming language for ordinary missed/unresolved events.

Experience direction

Use a calm, high-contrast visual system. Prioritize a patient home screen with today's schedule, next dose, clear status labels and a prominent confirmation action. Keep advanced analytics and configuration one level deeper. Support screen readers, dynamic text sizing and non-color status indicators.

6. Feature requirements and prioritization

Priority definitions: P0 = required for the MVP end-to-end journey; P1 = valuable next iteration or stretch goal; P2 = later roadmap / validation required.

ID	Feature	Priority	Requirement / behavior
FR-01	Account and profile	P0	Patient can register, sign in, manage profile and choose language/accessibility preferences.
FR-02	Medication records	P0	Patient can add, view, edit and archive a medication record; retain change history.
FR-03	Schedule configuration	P0	Create dose times/frequency from verified instructions; preserve prescribed text separately from reminder settings.
FR-04	Reminder delivery	P0	Schedule local reminders; show permission status and explain when OS settings may block delivery.
FR-05	Dose event logging	P0	Record taken, skipped/not taken, or unresolved; capture timestamp and source.
FR-06	Dose history	P0	View daily/weekly history with explicit status and correction trail.
FR-07	Adherence dashboard	P0	Display medication-wise and overall trends with clear denominator and date range.
FR-08	Caregiver invitation	P0	Patient invites caregiver and grants scoped permissions; caregiver must accept.
FR-09	Caregiver notifications	P0	Send only patient-authorized alerts/summaries according to configured rules.
FR-10	Consent management	P0	Patient can review, change and revoke sharing; changes take effect promptly.
FR-11	Pattern insights	P1	Detect repeated unresolved or missed events by weekday/time; explain evidence.
FR-12	Patient-reported reasons	P1	Allow optional reason selection such as forgot, away from home, couldn't access medicine, side effect concern, or other.
FR-13	Prescription OCR	P1	Extract candidate fields for review; never activate a schedule without human verification.

ID	Feature	Priority	Requirement / behavior
FR-14	Refill tracking	P1	Optional stock estimate and user-configured refill reminder; clearly label estimates.
FR-15	PDF report export	P1	Export date-bounded patient-approved report with event definitions and caveats.
FR-16	Voice interaction	P2	Explore voice-based confirmation after accessibility and privacy testing.
FR-17	Predictive risk	P2	Only consider after sufficient validated data, governance and clinical review.

Expected / Asked / Wow feature framework

Layer	Feature examples	Product role
Expected	Medication schedules, reminders, taken/missed logging, history, basic dashboard, consent-based caregiver link	Table stakes; must be dependable
Asked	Flexible reminders, snooze, local language, offline use, large controls, OCR-assisted setup, refill tracking, PDF export	Respond to real friction and user requests
Wow	Explainable pattern detection, context-aware reminder preferences, patient-approved escalation, personalized summaries	Differentiation; validate before overbuilding

7. Core workflows and acceptance criteria

Workflow A: Add a medication

- Patient selects Add medication and enters the medication name and the instruction details they have from their prescription.
- Patient enters schedule/reminder preferences separately from the prescribed instruction.
- System shows a review screen and asks the patient to verify the entered details against the prescription or clinician/pharmacist instructions.
- Only after confirmation is the schedule activated. Ambiguous fields remain flagged and cannot silently become a schedule.

Acceptance criteria

- Every active medication has a name, schedule definition, effective date and source/verification status.
- Changing a reminder preference does not rewrite the prescribed instruction.
- Editing a schedule creates a new version; prior dose events remain tied to the version that generated them.
- Medication deletion archives the record and preserves historical event integrity.

Workflow B: Reminder and dose response

- At a scheduled time, the app creates a dose event and attempts a reminder.
- Patient can choose Taken, Skipped / Not taken, or Snooze / Remind me later.
- Taken records the patient-confirmed timestamp. Skipped records an explicit not-taken outcome and optional reason. Snooze changes reminder timing only, not the prescribed schedule.
- If no response arrives, the event remains Unresolved after its configured response window; it must not be silently classified as taken.

Acceptance criteria

- Event status and reminder delivery status are separate fields.
- Every dose response records timestamp, actor, source and schedule version.
- Duplicate taps or sync retries do not create duplicate dose events.

- Late responses are supported and visibly marked as late; the original scheduled time remains unchanged.

Workflow C: Invite a caregiver

- Patient enters caregiver contact details and selects the information to share.
- Caregiver receives an invitation and must authenticate/accept before access begins.
- Patient can change permissions or revoke access from a clear consent screen.
- Caregiver sees only the fields and events covered by the current permission.

Acceptance criteria

- No caregiver can access a patient record before acceptance and authorization.
- Revocation invalidates future access and active sessions/tokens according to security policy.
- Consent grants, changes and revocations are audit logged.
- Caregiver alerts are generated only for enabled event types and configured thresholds.

Workflow D: Review adherence insights

- Patient chooses a date range and sees scheduled events, confirmed taken events, explicit not-taken events and unresolved events.
- System presents simple trends, e.g. '3 unresolved morning doses on Mondays in the last 4 weeks'.
- Where available, patient-reported reasons are shown separately from system-detected patterns.
- Patient can dismiss an insight or mark it useful; no treatment advice is generated.

Acceptance criteria

- Every insight is traceable to specific dose events and a stated date range.
- Small sample sizes are labeled; no confident pattern is displayed from insufficient observations.
- Insight wording describes observed records and does not claim to know the cause.
- Data gaps and offline sync delays are disclosed in the dashboard.

8. Adherence intelligence and notification logic

MVP intelligence should be deterministic, explainable and auditable. Do not use a generative model to decide dose timing, interpret ambiguous prescriptions or make clinical recommendations.

Dose event state model

Concept	Meaning	Must not be confused with
Scheduled	A dose event is expected based on an active schedule version	Reminder sent
Reminder attempted	The app/notification service attempted delivery	Delivery confirmed
Delivered (if known)	Platform reports successful delivery where telemetry exists	Patient saw or acted on it
Acknowledged	Patient interacted with the reminder	Medication taken
Taken (patient-confirmed)	Patient explicitly recorded intake	Clinically verified ingestion
Not taken / skipped	Patient explicitly recorded that dose was not taken	Unresolved
Unresolved	No reliable outcome has been recorded by the reporting cutoff	Not taken
Snoozed	Patient asked to be reminded later	Schedule changed

Initial pattern rules

- Repeated unresolved/not-taken events in a particular time window across a rolling period.
- Day-of-week concentration, only when enough observations exist.
- Change in confirmed-dose proportion compared with the prior equivalent period.
- Repeated snoozes before a confirmed response.
- Patient-reported reason frequency, shown as self-reported rather than inferred.

Insight thresholds (initial configurable defaults)

Use a minimum evidence threshold before showing a pattern, such as at least 4 comparable scheduled events across at least 2 weeks. Treat this as a product hypothesis to validate, not a clinical threshold. Show exact counts and dates, and allow users to dismiss insights.

Reminder policy

- Patient configures reminder windows, quiet hours, snooze interval and maximum repeat count.
- Do not send repeated alerts indefinitely. Respect OS-level notification permissions and patient preferences.
- Escalation to a caregiver is disabled by default and requires explicit patient configuration and consent.
- Caregiver alert copy should avoid diagnosis or medical urgency claims; it should report only the configured event and timestamp.
- Never instruct a patient to double a dose, take a late dose, stop medication or change timing. Direct medication questions to their clinician or pharmacist.

9. Data model and technical architecture

Recommended MVP architecture

Layer	Recommended choice	Responsibilities
Patient mobile app	Flutter (Android-first)	Medication UI, local schedule cache, local notifications, dose event capture, offline queue
Web dashboards	React	Caregiver views, patient reports and future clinician-approved access
API/backend	FastAPI	Authentication integration, authorization, medication and event APIs, consent, report generation
Database	PostgreSQL	Users, medications, schedule versions, dose events, consent, notification records and audit events
Notifications	FCM plus platform/local scheduling	Push and local notification delivery; delivery telemetry where available
OCR (P1)	PaddleOCR or managed OCR	Candidate text extraction only; human verification required
Analytics	Python / SQL	Adherence aggregates, rule-based pattern detection and reporting
Deployment	Docker; managed hosting for demo	Repeatable deployment, backups, secrets and environment separation

Core entities

Entity	Important fields
User	id, role, display_name, locale, accessibility_preferences, auth_subject
PatientProfile	user_id, timezone, reminder_preferences
Medication	id, patient_id, display_name, prescribed_instruction_text, verification_status, active_status
ScheduleVersion	id, medication_id, frequency_rule, dose_times, effective_from, effective_to, created_by
DoseEvent	id, schedule_version_id, scheduled_at, status, responded_at, response_source, reason_code
ReminderAttempt	id, dose_event_id, attempted_at, delivery_status, channel
CareRelationship	id, patient_id, caregiver_id, relationship_status
ConsentGrant	id, patient_id, caregiver_id, scopes, alert_preferences, granted_at, revoked_at
Insight	id, patient_id, rule_id, evidence_event_ids, date_range, generated_at, dismissed_at
AuditEvent	id, actor_id, action, resource_type, resource_id, timestamp, outcome

Architecture and engineering requirements

- Use server-authoritative identity and authorization checks for every protected API request.
- Store timestamps in UTC and preserve the patient's IANA time zone for schedule interpretation and display.
- Use idempotency keys for dose-event writes and sync operations.
- Keep a local cache of active schedules and pending dose responses for offline operation.
- Resolve sync conflicts without silently overwriting a patient-confirmed event; preserve correction history.
- Separate personally identifiable information from analytics exports where practical.
- Back up the database and test restore procedures before any real-patient pilot.

10. Privacy, security and clinical safety

This product handles sensitive health information. Privacy and safety requirements are release blockers, not future enhancements.

Privacy and consent

- Collect only data needed for medication scheduling, dose tracking, support and reporting.
- Caregiver sharing is opt-in, purpose-specific, granular and revocable.
- Explain what each permission exposes before the patient grants it.
- Provide access, correction and deletion workflows consistent with applicable law and retention obligations.
- Do not sell health data or use identifiable patient data for advertising.
- Do not expose patient health content to support staff by default; use audited, time-limited break-glass access only if operationally necessary.

Security controls

- TLS for data in transit and encryption at rest for database, backups and file storage.
- Secure authentication, session expiry, rate limiting and protection against credential abuse.
- Role-based and resource-level access control; test for insecure direct object references.
- Secrets stored outside source code; separate development, staging and production credentials.
- Audit logging for sensitive access, permission changes, exports and administrative actions.
- Secure file handling for prescription images: access controls, retention limits, malware scanning where appropriate.

Clinical safety guardrails

- Never calculate or recommend a replacement dose after a missed dose.
- Never change dose, frequency, treatment duration or medication selection.
- Prescription OCR output is untrusted until verified by a human against the source.
- Label patient-confirmed intake as self-reported, not clinically verified.
- Do not infer reasons for missed medication; show only patient-entered reasons or neutral event patterns.
- Use clear copy directing medication questions to a clinician or pharmacist.
- Conduct accessibility, usability and safety testing before any real-world deployment.

India-specific compliance workstream

Before handling real patient data, obtain qualified legal and security review for applicable Indian requirements, including the Digital Personal Data Protection Act and rules in force at deployment, relevant health-data obligations, consent requirements, data retention, breach response and any applicable clinical software or medical-device classification. This PRD is not a legal opinion.

11. MVP scope, roadmap and delivery plan

Phase	Scope	Exit criteria
0 · Discovery	Patient/caregiver interviews, workflow mapping, usability assumptions, consent model	Validated top pain points; agreed terminology; prioritized MVP
1 · Foundation	Authentication, patient profile, medication records, schedule versions	Patient can create and verify a medication schedule
2 · Core loop	Dose events, local reminders, snooze, history, offline queue	End-to-end reminder → response → history works reliably
3 · Support	Caregiver invitation, granular consent, authorized summaries	Access tests pass; revocation and audit trail verified
4 · Intelligence	Adherence aggregates, simple patterns, explainable insights	Insights trace to source events and disclose uncertainty
5 · Pilot readiness	Accessibility, security review, backup/restore, device testing, feedback	No critical safety/privacy issues; pilot protocol approved

Recommended hackathon demo scope

- One patient account with 2–3 sample medications and configurable schedules.
- Reminder simulation with Taken, Skipped and Snooze actions.
- Daily/weekly adherence dashboard that distinguishes taken, not taken and unresolved.
- One rule-based insight backed by visible event history.
- Caregiver invitation/consent flow and a simulated authorized summary.
- Seeded demo data clearly labeled as synthetic; no real patient information.

Defer from hackathon MVP

- Predictive AI or machine-learning risk scores.
- Automatic interpretation of complex prescriptions.
- Hospital/EHR integration and pharmacy ordering.
- Emergency escalation, clinical triage or medical recommendations.
- Production-grade multi-tenant clinician workflows.

12. Risks, assumptions and validation plan

Risk / assumption	Why it matters	Mitigation / validation
Reminder delivery is not guaranteed	OS battery policies, permissions and connectivity can block notifications	Local scheduling, permission diagnostics, device test matrix, clear status semantics
Patients may not want caregiver alerts	Over-alerting can harm trust and autonomy	Interview patients; opt-in granular settings; alert preview and easy revoke
Manual data entry is burdensome	Setup friction can cause abandonment	Test onboarding; templates only if safe; OCR-assisted entry with human verification
Adherence metrics may mislead	Missing records are not proof of missed medication	Show unresolved separately; define denominator and reporting window
Pattern insights may overstate causality	Correlation does not explain why a dose was missed	Use descriptive language, evidence counts and patient-reported reasons
Health data exposure	Sensitive information can harm users	Least privilege, encryption, audit, security review and incident response
Clinical scope creep	Users may interpret insights as medical advice	Strict product copy, safety review, no dosage/treatment recommendations
Low-end devices / poor network	Target users may be excluded	Android device testing, offline-first core flow, lightweight UI

Key assumptions to test

- Patients value a simple and reliable reminder + confirmation loop more than complex AI features.
- Some patients want caregiver support, but want control over exactly what is shared.
- Patients can and will provide enough event feedback to make basic pattern summaries useful.
- Offline access and local notification reliability are important for the target population.
- Patient-friendly reports can support, but do not replace, clinical conversations.

Discovery research plan

Research activity	Suggested sample	Questions to answer
Patient interviews	8–12 chronic patients across age and digital-literacy levels	How do they currently remember, track and recover from missed doses?
Caregiver interviews	4–6 family caregivers	What information helps them support without over-monitoring?
Clinician/pharmacist review	3–5 professionals	Which adherence information is useful and what could be unsafe or misleading?
Usability test	5–8 target users	Can users add a schedule, respond to a reminder and find their history without assistance?
Device/field test	A mix of low- and mid-range Android devices	Do reminders and offline logs behave reliably under real constraints?

13. Competitive landscape and research notes

The desk research identified established medication-management products including Medisafe, MyTherapy, CareClinic and Dosecast. These products demonstrate that reminders, medication lists, dose tracking and health records are established market expectations. The proposed product should not claim those capabilities as unique.

Product	Observed positioning / capabilities to investigate	Design lesson
Medisafe	Medication schedules, reminders, dose tracking, adherence reports and caregiver/Medfriend features	Study reminder flows and caregiver setup; differentiate through transparent pattern insights and patient-controlled permissions
MyTherapy	Medication reminders and tracking alongside broader health measurements	Keep medication adherence workflow focused and reduce setup burden
CareClinic	Health management, symptom tracking, medication and reports	Avoid feature sprawl; make medication outcomes easy to understand
Dosecast	Flexible medication reminder and scheduling workflows	Study schedule flexibility and reminder configuration

Research interpretation and limitations

The initial desk research referenced a small Medisafe usability study (30 participants), app-review analysis and literature reviews on medication adherence applications. These findings are useful for generating hypotheses, but they are not representative market-wide prevalence estimates. The cited usability percentages should not be generalized to all patients or all medication apps.

The reported themes included notification fatigue, setup friction, technical/notification problems, accessibility barriers, limited use of caregiver features and requests for simpler workflows, local storage, better communication and useful exports. Validate each theme with target users before treating it as a product requirement.

Competitor feature availability changes by platform, geography and subscription tier. This comparison is directional and should be refreshed through hands-on product audits before a public competitive claim is made.

Reference products

- Medisafe: https://www.medisafe.com/
- MyTherapy: https://www.mytherapyapp.com/
- CareClinic: https://careclinic.io/
- Dosecast: https://www.dosecast.com/

Evidence references to verify before external publication

- Medisafe usability study cited in the preliminary research (2025; n=30). Verify full bibliographic details, sample characteristics and exact outcome definitions.
- Systematic review/meta-analysis of mobile applications and medication adherence cited in preliminary research. Verify study inclusion criteria and certainty of evidence.
- Asian medication-adherence app review and rural Maharashtra medication reminder study cited in preliminary research. Verify publication details and transferability to the intended users.

Note: The preliminary research citations were supplied in the preceding research response. This PRD preserves their findings as provisional desk-research inputs; full bibliographic metadata and source-level verification should be completed before submission, publication or clinical use.

14. Appendix: glossary and open decisions

Term	Definition
Scheduled dose event	An expected dose occurrence generated from an active schedule version.
Reminder attempt	An attempt to notify the patient; it does not prove delivery or intake.
Patient-confirmed dose	A dose the patient explicitly marks as taken; self-reported, not clinically verified.
Not taken / skipped	An explicit patient response that the scheduled dose was not taken.
Unresolved	No reliable dose outcome has been recorded by the reporting cutoff.
Adherence rate	A calculated measure whose numerator, denominator, exclusions and date range must be shown.
Caregiver scope	The exact categories of patient data a caregiver is authorized to access.
Schedule version	A dated version of medication reminder timing/instructions used to preserve history.

Open product decisions

- Final product name and brand identity.
- Initial target condition(s) and patient age range for user research.
- Android-only MVP versus Flutter cross-platform release.
- Exact schedule and response-window semantics, reviewed with a pharmacist/clinician.
- Supported languages for the first pilot.
- Whether clinician-facing dashboard is in scope or PDF export is sufficient.
- Data hosting region, retention period and legal basis for each processing purpose.
- Whether any real-patient pilot is feasible within the project timeline and governance constraints.

Definition of MVP done

- A patient can create and verify at least one medication schedule.
- A scheduled reminder can be triggered and its attempt/status recorded accurately.
- A patient can record Taken, Not taken or Snooze; unresolved remains distinct.
- History and adherence calculations are traceable to dose events and show date range/denominator.
- A patient can invite a caregiver, grant scoped consent and revoke access.
- Caregiver access is enforced server-side and tested against unauthorized access.
- Core schedule and dose logging remain usable offline and sync without duplicate events.
- No feature changes prescribed treatment or provides missed-dose medical advice.
- Critical accessibility, privacy and security checks are completed for the intended demo/pilot scope.

End of PRD · PS-05 Medication Adherence Intelligence Platform

Prepared as a product planning baseline. Revise this document after user discovery, clinician/pharmacist review, security assessment and prototype testing.